

DEVELOPMENTAL-BEHAVIORAL PEDIATRICS

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DEVELOPMENTAL-BEHAVIORAL PEDIATRICS NOTE

Patient: Marcus D. Whitfield MRN: PED-2025-04471

DOB: 03/03/2016 Age: 9 years, 6 months Sex: Male

Date of Visit: 09/15/2025 Visit Type: ADHD follow-up / developmental-behavioral follow-up

Accompanied By: Foster parent

Attending: Mei Liang, MD — Developmental Pediatrics

HISTORY OF PRESENT ILLNESS

Marcus is a 9-year-old boy seen today for follow-up of ADHD symptoms in the setting of neurodevelopmental complexity. He carries diagnoses of ASD Level 2, ADHD Combined Type, PTSD, and FASD. Caregiver reports that he continues to have significant distractibility, impulsivity, difficulty remaining seated, and need for frequent redirection across home and school settings. Mornings are generally more manageable after medication administration, with increased loss of focus and irritability in the late afternoon and evening, suggestive of stimulant wearing-off effect. No new tics have been observed. Appetite continues to be poor, particularly around midday, which caregiver attributes to the stimulant medication. Sleep initiation remains difficult despite melatonin and clonidine; the caregiver describes a typical latency of 45 to 60 minutes. He is attending school with IEP accommodations at Roosevelt Elementary and has a 1:1 aide during transitions.

CURRENT MEDICATIONS

- Methylphenidate ER 27 mg QAM
- Risperidone 1.5 mg QHS (managed by Dr. Osei, Psychiatry)
- Clonidine 0.1 mg QHS (managed by Dr. Osei)
- Budesonide/Formoterol 80/4.5 mcg BID (PCP)
- Albuterol MDI PRN
- Ferrous sulfate 325 mg daily (GI)
- Melatonin 3 mg QHS (OTC)
- PEG 3350 daily

EXAMINATION

General: Thin, cooperative male. Weight 25.4 kg (<5th percentile), Height 131.5 cm (10th percentile). BMI 14.7 (<5th percentile, underweight). Neurologic: Alert, oriented to person and place. Speech limited in volume and length of utterance, primarily using 2-to-3-word phrases with occasional echolalia. Receptive language appears stronger than expressive language. Stereotypic hand-flapping present throughout the visit. Eye contact limited and fleeting. Motor: Bilateral toe walking observed during ambulation in the clinic hallway — persistent pattern, reverts within several steps of cueing to walk flat. Fine motor skills grossly intact for age on screening tasks (pencil grip, button manipulation). Sensory: Covers ears intermittently in response to ambient clinic noise. Growth: Interval measurements reviewed; pattern notable for progressive growth decline over the past 12 months with height dropping from approximately 25th to 10th percentile.

ASSESSMENT

1. ADHD, combined presentation — partial symptomatic benefit on current stimulant regimen. Classroom reports indicate improved focus during morning hours with afternoon deterioration. No dose increase recommended at this time given growth concerns.
2. ASD Level 2 with associated communication and behavioral challenges requiring ongoing school and therapeutic supports.
3. Growth decline in the setting of stimulant treatment, limited dietary intake, and possible GI malabsorption from suspected Crohn's disease. Growth trajectory is concerning and requires coordinated monitoring across PCP, GI, and this office.
4. Bilateral toe walking — persistent, likely multifactorial (sensory processing in ASD, possible mild gastrocnemius tightness).

PLAN

- Continue methylphenidate ER 27 mg QAM. No dose increase at this time.

- Reviewed nutrition strategies with caregiver, including breakfast before medication when possible, calorie-dense evening intake, and structured snack opportunities after school.
- Discussed school supports, classroom structure, and behavioral interventions to reduce task avoidance. IEP goals appear appropriate; caregiver to monitor implementation at Roosevelt Elementary.
- Discussed risks and benefits of current stimulant regimen in the context of growth concerns. Caregiver wishes to continue current regimen for now.
- Bilateral toe walking noted; physical therapy referral placed for gait evaluation and possible AFO assessment. Referral faxed to Cook County Children's Rehabilitation Services.
- Follow-up in 6 months for ADHD reassessment, growth monitoring, and review of PT progress.

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Electronically signed: 09/15/2025 at 15:30 CST